

Is the primary endurance limiting impairment Reversible or irreversible?	Due to disease specific impairments?	
	Yes - Cardiopulmonary Disease Specific Impairments	No - Non – Cardiopulmonary Disease Specific Impairments
Reversible	Quadrant A Goals: Expect increased capacity Intervention: Focused on increasing capacity and re-evaluating once disease specific impairment is optimized. Expect changes in exercise prescription parameters frequently to continue to accommodate increased capacity Likely scenario: New medications or procedures; reversible conditions that exacerbate underlying chronic conditions (pneumonia, acute bronchitis, fluid loaded)	Quadrant B* Goals: Expect increased capacity Intervention: Initially expect increased capacity quickly and therefore expect to change exercise prescription parameters; will plateau unless reaches new thresholds due to increased muscular strength (in patients that start off very limited due to muscle weakness) Likely scenario: Muscular deconditioning (either primarily poor strength or primarily poor endurance to start)
Irreversible	Quadrant C* Goals: Expect slow gains, if gain at all in endurance; related to efficiency, energy conservation or increased tolerance Intervention: Focused on improving efficiency (if possible^), on energy conservation or increasing tolerance. Likely scenario: NYHA Class 3 – 4 CHF; end stage COPD with immediate reductions in SpO2 upon exertion, unable to change	Quadrant D Goals: Expect slow gains, if gain at all in endurance; related to efficiency, energy conservation or increased tolerance Intervention: Focused on improving efficiency, on energy conservation or increasing tolerance Likely scenario: co-morbidity that impacts endurance such as multiple sclerosis